

# AFib Anticoagulation Restart Timing After Acute Ischemic Stroke

Evidence-based schedule for starting or restarting DOACs after acute ischemic stroke or TIA.

## CLINICAL EFFICACY & SAFETY

RCT and individual-patient meta-analysis data support early DOAC initiation in carefully selected AFib-related ischemic stroke patients, especially mild-to-moderate infarcts without high-risk hemorrhagic transformation. Early treatment has not shown excess symptomatic intracranial hemorrhage (sICH) versus delayed treatment and may reduce recurrent ischemic stroke; DOACs are preferred over warfarin for most nonvalvular AF patients when anticoagulation is indicated.

### 1. Stroke Severity Classification (ELAN Imaging Criteria)

#### Minor / Small Infarct

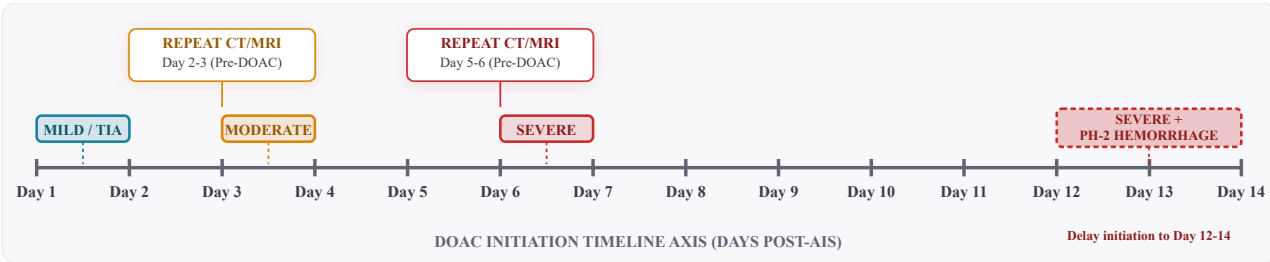
- TIA or infarct  $\leq 1.5$  cm on brain imaging. NIHSS can guide bedside risk but is not the ELAN definition.

#### Moderate Infarct

- Cortical superficial-branch lesion, internal border-zone lesion, or deep-branch lesion  $> 1.5$  cm.

#### Major / Large Infarct

- Complete vascular territory,  $\geq 2$  moderate lesions, large multilobar infarct, or brainstem/cerebellar lesion  $\geq 1.5$  cm.



### 2. Bedside DOAC Dosing & Adjustment Guide

| Drug                  | Standard Dose           | Dose Reduction Criteria                                                                                                                                       |
|-----------------------|-------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Apixaban (Eliquis)    | 5 mg BID                | <b>Reduce to 2.5 mg BID</b> if $\geq 2$ criteria are met:<br>• Age $\geq 80$ years   • Weight $\leq 60$ kg   • Serum creatinine $\geq 1.5$ mg/dL              |
| Rivaroxaban (Xarelto) | 20 mg daily (with food) | <b>Reduce to 15 mg daily</b> if CrCl is 15–50 mL/min.<br><b>Hold</b> if CrCl $< 15$ mL/min.                                                                   |
| Dabigatran (Pradaxa)  | 150 mg BID              | <b>Reduce to 75 mg BID</b> if CrCl is 15–30 mL/min.<br><b>Avoid</b> if CrCl $< 15$ mL/min.                                                                    |
| Edoxaban (Savaysa)    | 60 mg daily             | <b>Reduce to 30 mg daily</b> if CrCl is 15–50 mL/min or weight $\leq 60$ kg.<br><b>Avoid</b> if CrCl $> 95$ mL/min (high renal clearance reduces DOAC level). |

**ELAN Trial:** Fischer U et al. *N Engl J Med.* 2023;388:2411-2421. | **CATALYST Meta-Analysis:** Dehbi HM et al. *Lancet* 2025. Early DOAC (median Day 2) vs delayed (median Day 7-8) showed no excess sICH (0.4% vs 0.4%) and fewer recurrent ischemic events in pooled data.

**AFib Guidelines:** Joglar JA et al. 2023 ACC/AHA/ACCP/HRS Guideline. *Circulation.* 2024;149:e1-e156.